Fern & Fine Print

INSURANCE EDUCATION, MINUS THE HEADACHE

a free mini-course

Insurance Without the Headache

A calm, plain-English guide to health and life insurance basics, with a worksheet for every lesson.



8 short lessons · printable worksheets · a 50-state help directory

By Suzelaine Similien · Fern & Fine Print · Educational only

What's inside

Go in order if you can. Do the worksheets in pencil.

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How each lesson works. A short hook, the lesson, a one-page worksheet you can print, key takeaways, and the official sources, so you can check every fact yourself.

About this guide

Insurance Without the Headache is written by **Suzelaine Similien**, creator of Fern & Fine Print. Suzelaine is a licensed insurance agent (NPN 16685113). Fern & Fine Print is her separate education business. It does not sell, quote, or recommend insurance, and this guide is not an offer of insurance or a solicitation to buy any policy. We share this for transparency.

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Dates, dollar figures, and phone numbers were checked on official sites on Oct 29, 2026, and can change. Each lesson lists its sources.

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Lesson 0

Welcome: You're Not Behind

Reading time: about 2 minutes · Worksheet: 3 minutes



The hook

Ever typed "why is health insurance so complicated" into a search bar at 11:47 p.m.? You're in very good company. Questions like that, along with "how does health insurance work" and "what is a deductible," come up again and again in what people ask online.

So let's get one thing out of the way: **if insurance confuses you, that isn't a personal failing.** It's a product written in a dialect almost nobody speaks at home.

Insurance speaks fluent alphabet soup. You don't have to.

The lesson

What this course is

Think of it as a quiet room with a pot of tea and a highlighter. In eight short lessons we'll walk through:

#	Lesson	What you'll walk away with
0	Welcome (you are here)	A starting-point check, and how to use the course
1	What "coverage" actually means	The five money words, and the "sick-day cost" idea
2	When work insurance gets expensive (or ends)	The 60-day window and how COBRA works
3	Affordable vs. "covers everything"	The tradeoffs, the metal levels, and the core checklist
4	HMO vs. PPO in one coffee	Network types without the headache
5	Pre-existing conditions and plan types that surprise people	How to spot coverage that isn't what it looks like
6	Life insurance and your health coverage	AD&D, work life, graded benefits, and the fine print that links them to health coverage
7	Your next step	Real 2027 dates, free official help, and your one-page plan

Each lesson has the same parts: a short **hook**, the **lesson**, a **one-page worksheet**, **key takeaways**, and **sources**. The sources are there so you can check every fact we state, because you shouldn't have to take anyone's word for it.

What this course isn't

- It isn't a quote, a plan recommendation, or a sales pitch dressed up as a lesson.
- It isn't advice about your situation. Your best choice depends on your health, your budget, your doctors, and your state.
- It doesn't promise you'll save money or get approved for anything. Nobody honest can promise that.

The house rule

Whether you're looking at a free or low-cost option or you have more room in the budget, you get the same careful explanation and the same respect. You're a person, not a sale. This course is yours to keep, whatever you decide and wherever you end up buying.

How to use it

- **Go in order if you can.** Each lesson builds a little on the one before.
- **Do the worksheets in pencil.** They're meant to be messy. A half-filled worksheet is worth more than a perfect one you never started.
- **Come back to it.** Insurance questions tend to show up at life's turning points: a new job, a lost job, a baby, a move, a birthday ending in zero. The course will still be here.

Key takeaways

- Insurance is confusing because it's complicated, not because you're "bad at adulting."
- This course teaches. It doesn't sell, quote, or recommend a plan.
- Every lesson lists official sources so you can check the facts yourself.
- Same respect for every budget. That's the house rule.

Next up → Lesson 1: What "coverage" actually means.

Sources

- The example questions in the hook come from Fern & Fine Print's review of Google Autocomplete suggestions on Oct 29, 2026. Autocomplete shows what people type, not how often they type it.
- HealthCare.gov, "Health Insurance Basics" glossary (the terms we'll use in Lesson 1): healthcare.gov/glossary

Worksheet · "Where am I starting?"

Print this page or copy it into your notes. There are no wrong answers.

1. Right now, my health coverage comes from: (circle one) Work · A family member's plan · A plan I bought myself · Medicare · Medicaid/CHIP · I'm not sure · I don't have coverage right now

2. The reason I'm here is mostly: (circle any) Understanding the words · A job change · Open Enrollment is coming · A new family member · Life insurance questions · Just want to feel less lost

3. Rate your confidence, 1 (lost in the woods) to 5 (could teach it):

I can explain...	1	2	3	4	5
What a deductible is	☐	☐	☐	☐	☐
What an out-of-pocket maximum is	☐	☐	☐	☐	☐
The difference between an HMO and a PPO	☐	☐	☐	☐	☐
When I'm allowed to sign up for or change a plan	☐	☐	☐	☐	☐
Term vs. permanent life insurance	☐	☐	☐	☐	☐

4. The one insurance question that bugs me most:

_____

5. Dates I already know I need to remember (a job ending, a birthday, a renewal letter):

_____

At the end of Lesson 7 you'll rate yourself again. Watching the numbers move is very satisfying.

Lesson 1

What "Coverage" Actually Means

Reading time: about 4 minutes · Worksheet: 5 minutes



The hook

"What is a deductible?" "What is coinsurance?" "What is an out-of-pocket maximum?" These are some of the most common insurance questions people type into a search bar. That makes sense, because these five little words decide what you actually pay.

Here's the twist: **the cheapest plan every month isn't always the cheapest plan in the month you need care.** By the end of this lesson you'll know how to look at both.

The lesson

The five money words

Picture your plan as a garden gate with a few different latches.

1. Premium. What you pay every month to keep the plan, whether you use care or not. HealthCare.gov's glossary points out that besides the premium, you usually also pay other costs when you get care: a deductible, copays, and coinsurance.

2. Deductible. What you pay for covered services before the plan starts paying its share. HealthCare.gov's example: with a $2,000 deductible, you pay the first $2,000 of covered services yourself. Some helpful fine print:

- Marketplace plans pay the full cost of certain **preventive services** even before you meet your deductible, as long as you see an in-network provider. (HealthCare.gov notes that $0 isn't guaranteed in every case, so check.)
- Many plans pay for some services, like a checkup, before the deductible is met.
- Some plans have a **separate deductible** for prescription drugs.
- Family plans often have both an individual deductible and a family deductible.

3. Copayment ("copay"). A **flat amount**, like $20, that you pay for a covered service. Copays can be different for different services in the same plan (a doctor visit vs. a specialist vs. a lab test). Check your plan to see whether a given copay applies before or after the deductible.

4. Coinsurance. A **percentage** of the cost that you pay after you've met your deductible, like 20%. HealthCare.gov's example: if the allowed amount for a visit is $100 and your coinsurance is 20%, you pay $20 once your deductible is met. Before that, you pay the full $100.

> **Copay vs. coinsurance in one line:** a copay is a price tag ($20). Coinsurance is a slice of the pie (20%).

5. Out-of-pocket maximum (or limit). The most you'll pay for covered, in-network services in a plan year. After you reach it, the plan pays 100% of covered benefits for the rest of that plan year. HealthCare.gov lists what does **not** count toward it:

- your monthly premiums
- services the plan doesn't cover
- out-of-network care
- amounts above what the plan allows for a service

For Marketplace plans, the law caps how high this limit can be. HealthCare.gov lists the caps as:

Plan year	Individual	Family
2026	$10,600	$21,200
2027	$12,000	$24,000

Those are the highest a Marketplace plan's limit is allowed to be. Many plans set theirs lower. Your plan's own number is the one that matters, and it's listed in its Summary of Benefits and Coverage. (Figures checked on HealthCare.gov Oct 29, 2026.)

Putting it together: a worked example

This is HealthCare.gov's own illustration of a serious, expensive year, with simple round numbers:

- Allowed costs for the year: **$12,000**
- Deductible: **$3,000**
- Coinsurance: **20%**
- Out-of-pocket maximum: **$6,850**

1. You pay the first **$3,000** (the deductible).
2. You pay 20% of the remaining $9,000, which is **$1,800** (coinsurance).
3. Total: **$4,800**, which is under the $6,850 cap. If costs had kept climbing, you'd stop paying at $6,850 for covered in-network care, and the plan would cover the rest of the plan year.

And don't forget the premiums. You pay those every month on top of all this.

The "sick-day cost" idea

HealthCare.gov says it plainly: the plan with the lowest monthly premium may not be the best match for you. If you need a lot of care, a plan with a slightly higher premium and a lower deductible could cost you less overall.

So when you compare plans, look at two numbers, not one:

- **The quiet-year cost:** premium × 12 (plus a few copays)
- **The rough-year cost:** premium × 12 **plus** the out-of-pocket maximum

Neither number is a prediction. Together they show you the range a plan could cost you, from a quiet year to a rough one.

Key takeaways

- **Premium** keeps the plan. **Deductible**, **copay**, and **coinsurance** are what you pay when you use care.
- **Copay = a flat amount. Coinsurance = a percentage.**
- The **out-of-pocket maximum** caps your yearly costs for covered, in-network care. It doesn't include premiums, out-of-network care, or services the plan doesn't cover.
- For 2027, Marketplace plans can't set that limit above **$12,000** for an individual or **$24,000** for a family (HealthCare.gov).
- Compare both the **quiet-year** and **rough-year** cost, not just the monthly price.

Next up → Lesson 2: When work insurance gets expensive (or ends).

Sources

- HealthCare.gov glossary, Premium: healthcare.gov/glossary/premium
- HealthCare.gov glossary, Deductible: healthcare.gov/glossary/deductible
- HealthCare.gov glossary, Copayment: healthcare.gov/glossary/co-payment
- HealthCare.gov glossary, Coinsurance (including the $12,000 worked example): healthcare.gov/glossary/co-insurance
- HealthCare.gov glossary, Out-of-pocket maximum/limit (2026 and 2027 caps; checked Oct 29, 2026): healthcare.gov/glossary/out-of-pocket-maximum-limit
- HealthCare.gov, Preventive health services: healthcare.gov/coverage/preventive-care-benefits

Worksheet · "My sick-day cost"

Use one column per plan you're looking at, including your current plan. Find the numbers in each plan's Summary of Benefits and Coverage (SBC).

	Plan A	Plan B	Plan C
Plan name			
Monthly premium	$	$	$
× 12 = yearly premium	$	$	$
Deductible (individual / family)	$	$	$
Separate drug deductible? (Y/N, amount)			
Primary-care visit: copay or coinsurance?			
Coinsurance after deductible	%	%	%
Out-of-pocket maximum	$	$	$
Quiet-year cost (yearly premium)	$	$	$
Rough-year cost (yearly premium + OOP max)	$	$	$

Gut-check questions:

- ☐ In a rough year, could I cover the out-of-pocket maximum? If not, what would I do?
- ☐ Do I expect regular care (prescriptions, therapy, a planned procedure)? If so, lean on the rough-year column.
- ☐ Did I check whether my regular visits are covered **before** the deductible?

My plain-English definitions (write them in your own words; it really helps them stick):

Deductible = _____

Coinsurance = _____

Out-of-pocket max = _____

Lesson 2

When Work Insurance Gets Expensive (or Ends)

Reading time: about 4 minutes · Worksheet: 5 minutes



The hook

COBRA sounds dramatic because the bill often is.

When job coverage ends, two questions tend to show up at once: *"What are my options?"* and *"How long do I have?"* The good news is that there's usually a window. The less-good news is that the window has an end date, and it starts ticking before most people open their mail.

The lesson

First, the job plan that feels like a second rent

Even before coverage ends, many people notice their share of the premium creeping up year after year. If that's where you are, Lessons 1 and 3 will help you compare your options. This lesson is about the moment coverage actually **changes or ends**: a layoff, a quit, reduced hours, a divorce, or aging off a parent's plan at 26.

The two main paths, per HealthCare.gov

When you lose job-based coverage, HealthCare.gov describes **two main options**:

Path 1: A Marketplace plan, through a Special Enrollment Period (SEP)

- Losing job-based coverage qualifies you for a Special Enrollment Period. That's true **even if you quit or were fired**.
- You generally have **60 days after losing coverage** to pick a plan. You can also qualify if you **expect** to lose coverage in the next 60 days, so you can line things up ahead of time.
- Marketplace coverage can't start the same day job coverage ends. It starts **the first day of the month after**. HealthCare.gov's example: if coverage ends March 7 and you pick a plan by March 31, the new plan can start April 1.
- When you apply, you'll find out whether you qualify for savings based on your **estimated income for the whole calendar year** (including what you earned before the job ended), or for Medicaid/CHIP.
- You may need to send documents proving you lost coverage.

Path 2: COBRA continuation coverage

- COBRA lets you keep **the same job-based plan** for a limited time. It's usually **18 months** after a job ends or hours are cut, and longer in some situations, such as divorce or the death of the covered employee.
- The catch: you usually pay **the full premium yourself**, including the part your employer used to pay. The U.S. Department of Labor says the charge can be **up to 102%** of the plan's cost (the full premium plus 2%).
- COBRA generally applies to employers with **20 or more employees**. Many states have their own "mini-COBRA" rules for smaller employers, so check with your state's insurance department (every state's is listed in the Find Your State appendix).
- You'll get an **election notice**, and you have **at least 60 days** to decide. After you elect, you have **at least 45 days** to make the first payment (DOL).
- Your deductible, copays, and doctor network generally **stay the same**, because it's the same plan.

The fine print people trip on

- **Dropping COBRA early on purpose usually doesn't open a new window.** HealthCare.gov says voluntarily stopping COBRA payments doesn't qualify you for a Special Enrollment Period. DOL adds that if you end COBRA early, you generally can't enroll in Marketplace coverage until the next Open Enrollment. When COBRA **runs out** on its own, you get 60 days to choose a Marketplace plan.
- **A spouse's or parent's job plan may be an option too,** but often with a shorter clock. DOL notes you usually have **only 30 days** to request special enrollment in another group plan.
- **Starting a new job with a waiting period?** HealthCare.gov says a Marketplace plan can bridge the gap, and you can end it without penalty when the job coverage starts.
- **Turning 26 on a parent's plan** also counts as a loss of coverage for a Special Enrollment Period.
- **Medicaid and CHIP** accept applications any time of year.

Don't panic-buy at midnight

The most useful thing you can do the day you learn coverage is ending is **write down the date it ends**. Every window in this lesson is counted from that date. Then compare your options while you're inside the window, not after it.

Key takeaways

- Losing job-based coverage, for any reason, generally opens a **60-day Special Enrollment Period** for a Marketplace plan. You can also qualify up to 60 days **before** the coverage ends.
- **COBRA** keeps the same plan, usually for **18 months**, but you can be charged **up to 102%** of the full cost.
- You have **at least 60 days** to elect COBRA and **at least 45 days** to make the first payment.
- **Voluntarily dropping COBRA** generally doesn't open a new enrollment window.
- Write down your **coverage end date** first. Every other date is counted from it.

Next up → Lesson 3: Affordable vs. "covers everything."

Sources

- HealthCare.gov, If you lose job-based health insurance (two options, 60 days, coverage start, March 7 example, income counting, new-job waiting period): healthcare.gov/have-job-based-coverage/if-you-lose-job-based-coverage
- HealthCare.gov, Special Enrollment Periods (past 60 / next 60 days, voluntary COBRA drop, turning 26): healthcare.gov/coverage-outside-open-enrollment/special-enrollment-period
- U.S. Department of Labor, Continuation of Health Coverage (COBRA) (up to 102%, 20+ employees): dol.gov/general/topic/health-plans/cobra
- U.S. Department of Labor, A Worker's Guide to Health Benefits Under COBRA (60-day election, 45-day first payment, 18/36 months, 30-day group special enrollment, mini-COBRA, ending COBRA early): dol.gov/agencies/ebsa/about-ebsa/our-activities/resource-center/publications/workers-guide-health-benefits-cobra

Worksheet · "My coverage-change timeline"

1. Key dates

Item	Date
Last day of my job-based coverage (ask HR; it's often the end of the month, not your last workday)	
+ 60 days = last day to pick a Marketplace plan through the SEP	
Date my COBRA election notice arrived	
+ 60 days (at least) = my COBRA election deadline	
If I'm switching to a spouse's or parent's job plan: their deadline (often 30 days)	

2. Questions to ask HR or the benefits office (tick when answered)

- ☐ On exactly what date does my coverage end?
- ☐ What would the monthly COBRA premium be for me / my family?
- ☐ Does any severance include help paying for COBRA?
- ☐ Will any work **life insurance** end too, and can I keep or convert it? *(More in Lesson 6.)*
- ☐ Is my HSA or FSA affected, and what's the deadline for any claims?

3. Side by side

	COBRA (same plan)	Marketplace plan	Spouse's / parent's plan
Monthly premium	$	$	$
Deductible already met this year carries over?	Usually yes (same plan)	☐ check	☐ check
My doctors in network?			
My prescriptions covered?			
Deadline to decide			

4. My next action and the date I'll do it: _____

Lesson 3

Affordable vs. "Covers Everything"

Reading time: about 4 minutes · Worksheet (the core checklist): 5 minutes

The hook

"Which health insurance is the best?" is one of the questions people ask most online. Here's the honest answer: **there's no "best" plan, only the best match for you.**

The unicorn plan, with a tiny premium, every doctor, every drug, and a tiny deductible, is lovely to imagine and very rare in the wild. Real shopping is about **tradeoffs**. Once you can see them, the choices stop feeling like a coin toss.

The lesson

The three big seesaws

1. **Wider network ↔ often a higher cost.** More doctors and hospitals to choose from usually means paying more for that freedom. *(Lesson 4 unpacks network types.)*
2. **Lower premium ↔ often a higher deductible.** HealthCare.gov's glossary says it directly: plans with lower monthly premiums generally have higher deductibles, copays, and coinsurance, and the reverse is also true.
3. **The drug list (formulary) matters a lot if you take prescriptions.** A formulary is the list of prescription drugs a plan covers. A plan that's great on paper can be expensive if your medication isn't on it, or sits in a costly tier.

The metal levels, decoded

Marketplace plans come in four "metal" categories. HealthCare.gov stresses that **the metal has nothing to do with the quality of care.** It describes how you and the plan split costs, on average:

Category	Plan pays (estimate)	You pay (estimate)	Deductible is generally
Bronze	60%	40%	High
Silver	70%	30%	Moderate
Gold	80%	20%	Low
Platinum	90%	10%	Low

These are averages across covered services, not what you'll pay for any single visit. Actual costs vary by plan.

A few more details from HealthCare.gov:

- **Silver has a secret door.** If your income qualifies you for "extra savings" (cost-sharing reductions), you only get them if you choose a **Silver** plan. With those savings, the plan's share can rise to an estimated 73–94%.
- **Premium tax credits** (help with the monthly premium), if you qualify, can be used with **any** metal level.
- **Catastrophic plans** are a fifth category, available to people **under 30** or those who qualify for certain hardship or affordability exemptions.
- **HSAs:** HealthCare.gov says **all Bronze and Catastrophic plans** work with a Health Savings Account, as do some plans in other categories. *(This is a recent change; checked Oct 29, 2026. HSA tax rules come from the IRS, so ask a tax professional about your situation.)*

What every Marketplace plan has to include

Whatever the metal, HealthCare.gov says all Marketplace plans must cover the same **10 essential health benefits**, including prescription drugs, emergency services, hospitalization, lab services, and mental health and substance use disorder services, plus **preventive services at no cost** from in-network providers.

So the difference between plans usually isn't *whether* something is covered. It's **how much you pay** when you use it, and **which doctors** you can use.

Your secret weapon: the Summary of Benefits and Coverage

Every plan has a **Summary of Benefits and Coverage (SBC)**, a standardized, easy-to-read summary built so you can compare plans apples to apples. When a brochure says "great coverage!", the SBC tells you what that actually means. Ask for it, and read it with Lesson 1's worksheet beside you.

You can also compare each Marketplace plan's **quality rating**, a 1–5 star score shown when you compare plans.

Key takeaways

- There's no "best" plan, only the best **match**. Shopping means choosing your tradeoffs on purpose.
- **Lower premium usually means higher costs when you get care**, and the reverse.
- **Metal levels show how costs are split, not the quality of care.**
- Extra savings on deductibles and copays, if you qualify, come **only with Silver** plans.
- All Marketplace plans cover the **10 essential health benefits**. The differences are in **cost** and **network**.
- Read the **Summary of Benefits and Coverage**. It's the most honest page in the stack.

Next up → Lesson 4: HMO vs. PPO in one coffee.

Sources

- HealthCare.gov, Health plan categories: Bronze, Silver, Gold & Platinum (percentages, Silver savings, Catastrophic, HSA note, 10 essential health benefits, star ratings; checked Oct 29, 2026): healthcare.gov/choose-a-plan/plans-categories
- HealthCare.gov glossary, Formulary: healthcare.gov/glossary/formulary
- HealthCare.gov glossary, Deductible / Copayment / Coinsurance (premium tradeoffs): healthcare.gov/glossary/deductible
- HealthCare.gov, Tips about the Health Insurance Marketplace (what all Marketplace plans cover): healthcare.gov/quick-guide/one-page-guide-to-the-marketplace
- HealthCare.gov, Private plans outside the Marketplace (Summary of Benefits and Coverage definition): healthcare.gov/private-plan-exceptions-outside-open-enrollment

Worksheet · The core checklist

This is the course's main checklist. Tick each item for every plan you're seriously considering. Print one copy per plan.

Plan name: _____

- ☐ My primary doctor is in network
- ☐ The specialists I need are in network (or I understand and accept the tradeoff)
- ☐ The hospital I'd want to go to is in network
- ☐ I can name the **deductible** and the **out-of-pocket maximum** without looking
- ☐ My medications are on the **formulary**, at a cost I can live with
- ☐ I've worked out the **quiet-year** and **rough-year** cost (Lesson 1 worksheet)
- ☐ I know which metal level this is, and if I qualify for extra savings, I checked Silver
- ☐ I read the **Summary of Benefits and Coverage**
- ☐ I know whether this plan is ACA-compliant *(see Lesson 5)*
- ☐ This fits my enrollment window or life event *(see Lessons 2 and 7)*
- ☐ I've thought about who depends on my income *(see Lesson 6)*

The one tradeoff I'm choosing on purpose: (e.g., "a higher deductible in exchange for a lower premium, because I rarely see a doctor")

_____

What would change my mind:

_____

Lesson 4

HMO vs. PPO in One Coffee

Reading time: about 3 minutes (about one medium latte) · Worksheet: 5 minutes



The hook

"HMO vs PPO" is one of the most-searched plan questions, with "PPO vs EPO" and "is a high deductible health plan good" close behind. Here's the whole story in one sentence:

Neither one is "the good one." A mismatch is what hurts.

The wrong network can mean your favorite doctor suddenly costs full price, or isn't covered at all. So let's match the plan type to the life you actually live.

The lesson

First, what's a "network"?

A network is the group of doctors, hospitals, pharmacies, and other providers a plan has contracts with. In network usually means you pay less. Out of network can mean you pay more, or that the plan pays nothing. Plan types are mostly different rules about **how much the network matters**.

The four plan types, per HealthCare.gov

HMO (Health Maintenance Organization)

- Usually limits coverage to doctors who work for or contract with the HMO.
- Generally **won't cover out-of-network care except in an emergency.**
- May require you to **live or work in its service area.**
- Often focuses on prevention and coordinated care, and many HMOs have you choose a primary doctor and get referrals for specialists. *(Check each plan's rules.)*
- *Often* comes with a lower premium. That's common, but not guaranteed.

PPO (Preferred Provider Organization)

- You pay less with in-network providers.
- You **can** see out-of-network doctors and hospitals **without a referral**, for an additional cost.
- That flexibility usually costs more, in the premium, at the visit, or both.

EPO (Exclusive Provider Organization)

- Services are covered **only** with in-network doctors, specialists, and hospitals, **except in an emergency.**

- Think of it as "network-only," a bit like an HMO. Each plan sets its own referral rules, so check.

POS (Point of Service)

- You pay less with in-network providers.
- You need a **referral from your primary care doctor to see a specialist.**
- A bit of a blend: the referral habit of an HMO with some of a PPO's flexibility.

The 10-second cheat sheet

	Out-of-network covered?	Referral to see a specialist?
HMO	Generally no (except emergencies)	Often yes
EPO	No (except emergencies)	Depends on the plan
POS	Usually, at a higher cost	Yes
PPO	Yes, at a higher cost	No

"Often" and "depends on the plan" are doing real work here. The plan's own documents always win.

And the HDHP?

A **High Deductible Health Plan** isn't a network type. It's a **cost design**, and it can come as an HMO, a PPO, and so on. HealthCare.gov describes it as a plan with a higher deductible than a traditional plan, usually with a lower monthly premium. It can be paired with a **Health Savings Account (HSA)**, which is why it's often called an "HSA-eligible plan." Is it "good"? It depends on your expected care, your cash cushion, and whether you'll actually use the HSA. That's Lesson 1's quiet-year/rough-year math again.

The list that does most of the sorting

Before comparing plans, make one list: **your must-keep doctors, your regular prescriptions, and your preferred hospital.** Then check each name in each plan's provider directory and formulary. That list rules plans in or out faster than any brochure.

> **Pro tip:** directories aren't always current. If a doctor really matters to you, call their office and ask, "Do you take *[exact plan name]* for *[the coming year]*?" Use the plan's exact name. "We take Brand X" doesn't necessarily mean they take every Brand X plan.

Key takeaways

- **HMO and EPO:** network-only, except emergencies. **PPO:** out-of-network allowed, at a higher cost, no referral needed. **POS:** in between, with referrals to specialists.
- **HDHP** is a cost design, not a network type, and can be paired with an **HSA**.
- Neither HMO nor PPO is "the good one." **Mismatch** is what hurts.
- Make your **must-keep list** of doctors, drugs, and hospital first. Check names, and call to confirm the exact plan.

Next up → Lesson 5: Pre-existing conditions and plan types that surprise people.

Sources

- HealthCare.gov, Health insurance plan & network types: HMOs, PPOs, and more (HMO, PPO, EPO, POS definitions): healthcare.gov/choose-a-plan/plan-types
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- HealthCare.gov glossary, Formulary: healthcare.gov/glossary/formulary

Worksheet · "My must-keep list"

1. My people and places

Name	Type (doctor / specialist / hospital / pharmacy)	Plan A in network?	Plan B in network?	Confirmed by phone?
		☐ Y ☐ N	☐ Y ☐ N	☐
		☐ Y ☐ N	☐ Y ☐ N	☐
		☐ Y ☐ N	☐ Y ☐ N	☐
		☐ Y ☐ N	☐ Y ☐ N	☐

2. My regular prescriptions

Medication	On Plan A's formulary? Tier?	On Plan B's formulary? Tier?

3. How I actually use care (circle)

I like going straight to specialists without a referral: **Yes / No / Don't care**

I travel or live part of the year somewhere else: **Yes / No**

I'm happy to have one primary doctor coordinate my care: **Yes / No**

I'd pay more each month for out-of-network flexibility: **Yes / No / Not sure**

4. Based on my answers, the plan type that seems to fit my life: HMO · EPO · POS · PPO · Still deciding

Lesson 5

Pre-Existing Conditions and Plan Types That Surprise People

Reading time: about 4 minutes · Worksheet: 5 minutes



The hook

"Can insurance deny you for a pre-existing condition?" "What are short-term health plans?" "Is accident insurance the same as health insurance?" These questions keep coming up for a good reason: **some things that look like health insurance don't work like health insurance.**

A low monthly price can hide a gap. This lesson gives you one question that clears up most of the confusion: *"Is this ACA-compliant, and what does it exclude?"*

The lesson

The good news: Marketplace plans and pre-existing conditions

According to HealthCare.gov:

- **No Marketplace plan can reject you, charge you more, or refuse to pay for essential health benefits** because of a condition you had before coverage started.
- **Once you're enrolled**, the plan can't deny you coverage or raise your rates based only on your health.
- **Pregnancy** is covered from the day the plan starts, and a plan can't reject you or charge you more because you're pregnant when you apply.
- **Medicaid and CHIP** also can't refuse to cover you or charge you more for a pre-existing condition.

The exception: "grandfathered" individual plans bought on or before March 23, 2010 don't have to cover pre-existing conditions or preventive care. If you have one of these, HealthCare.gov describes your options to switch.

"Can my health insurance just drop me?"

Under the ACA, **rescission** (an insurer cancelling your policy retroactively, back to the start) is illegal **except for fraud or intentional misrepresentation of a material fact.** That's one more reason to answer every application question completely and honestly. Coverage can still end for other reasons, like unpaid premiums, so keep an eye on due dates.

The surprise #1: short-term health plans

The National Association of Insurance Commissioners (NAIC) describes short-term, limited-duration plans this way:

- They're meant for **temporary** coverage and typically offer **limited benefits**.
- They're often **medically underwritten**, meaning the company can review your health history to decide whether to cover you and what to charge.
- They **may not cover pre-existing conditions** and **aren't required to cover essential health benefits** like maternity care, prescription drugs, or mental health care.
- They **can have annual and lifetime limits** on benefits.
- They often have lower premiums because they cover less and have fewer rules.

The federal rules on how long these plans can last have **changed several times** and were in flux as of mid-2026, and states set their own limits. Some states ban them outright. Check your state's current rules before relying on one. *(Status per NAIC, last updated July 2, 2026.)*

The surprise #2: "supplemental" or "extra" policies

Accident insurance, hospital indemnity, critical illness (sometimes called specified disease), and similar policies usually **pay a set cash amount when a specific thing happens**: an accident, a hospital stay, a listed diagnosis. They can help with costs like a deductible or lost wages.

Under federal law they're **"excepted benefits."** That means they're designed to supplement major medical coverage, they aren't subject to the ACA's market rules, and they **don't count as qualifying ("minimum essential") health coverage.** In plain English, they sit **alongside** a health plan. **They're not a replacement for one.**

Before buying any "extra," read the **benefit schedule** (what triggers a payout and how much it is), the **exclusions**, and any **waiting periods**. Don't rely on the brochure.

A quick word on dental

Per HealthCare.gov:

- **Dental is an essential health benefit for children, but not for adults.** Marketplace health plans don't have to offer adult dental.
- You can choose a health plan that includes dental, or buy a **separate dental plan**, but you can't buy a Marketplace dental plan without also buying a health plan.
- **Separate dental plans can have waiting periods** before they cover some services for adults, and you still pay premiums during the wait.

Look-alike websites

Michigan's insurance regulator warned in October 2026 that **not every site that looks official is HealthCare.gov.** Some are run by "lead generators" that collect your information and pass it to marketers, and some sell plans with limited benefits that don't qualify for financial help. Check the web address before you type anything personal.

Key takeaways

- **Marketplace plans can't reject you or charge you more for pre-existing conditions**, and they can't raise your rates based only on your health once you're enrolled.
- **Short-term plans** can exclude pre-existing conditions, skip essential benefits, and cap what they pay. State and federal rules keep changing.
- **Supplemental policies** (accident, hospital indemnity, critical illness) pay set cash amounts. They **sit alongside** a health plan and don't replace one.
- **Adult dental** isn't an essential health benefit, and separate dental plans can have **waiting periods**.
- Before you fall in love with a monthly number, ask: **"Is this ACA-compliant? What does it exclude?"**

Next up → Lesson 6: Life insurance and your health coverage.

Sources

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Worksheet · "Is this the real thing?" red-flag check

Use this for any policy you're offered, especially one with a surprisingly low price.

Policy name / type: _____

Question	Answer	Red flag if...
Is it an ACA-compliant major medical plan?	☐ Yes ☐ No ☐ Not sure	No or not sure
Does it cover pre-existing conditions?	☐ Yes ☐ No ☐ Limited	No or limited
Does it cover the 10 essential health benefits (including prescriptions, mental health, maternity)?	☐ Yes ☐ No ☐ Some	No or some
Is there an annual or lifetime dollar limit on benefits?	☐ Yes ☐ No	Yes
Does it pay a **fixed cash amount** rather than paying my medical bills?	☐ Yes ☐ No	Yes (it's supplemental, not a health plan)
Are there waiting periods? How long?		Long, or unclear
Did they ask health questions before quoting a price?	☐ Yes ☐ No	Yes (it's underwritten, so not ACA-style)
Can I see the full policy or Summary of Benefits **before** paying?	☐ Yes ☐ No	No
Am I on the real HealthCare.gov (or my state's official site)?	☐ Yes ☐ No ☐ N/A	No
Is someone pressuring me to decide today?	☐ Yes ☐ No	Yes

Number of red flags: _____ One flag isn't automatically a deal-breaker, but it does mean you should **understand exactly what you're giving up** before you pay.

The two questions I'll ask before I buy anything:

1. "Is this ACA-compliant?"
2. "What does it exclude?"

Lesson 6

Life Insurance and Your Health Coverage

Reading time: about 7 minutes · Worksheet: 10 minutes (with your benefits paperwork nearby)



The hook

"Does my health insurance come with life insurance?" People ask it a lot, especially around open enrollment, and the honest answer is **"not by itself, but life-type benefits often travel with it."**

They hide in job benefits, association packages, and supplemental policies, under names like AD&D, basic life, graded benefit, and living benefits. For people whose health history makes a traditional life policy hard to get, these can matter a lot. They also come with fine print. Let's find both.

The lesson

What it is, in one breath

Health insurance helps pay for care while you're here. **Life insurance pays money to the people you name, your beneficiaries, if you die while the policy is in force.** Traditional policies come in two families: **term** (a set period, generally the most protection per premium dollar) and **permanent** (meant to last, with a cash value, so premiums tend to be higher), per the NAIC.

Most individual policies are **underwritten**: the insurer reviews your health, may ask for an exam, and can charge more or decline you because of your health (Texas Department of Insurance). That's why the benefits below can matter so much, and why it pays to understand them.

First, the myth: "My health plan includes life insurance."

A health plan pays for medical care. **Life insurance isn't part of an ACA health plan's benefits**, and the Marketplace can only offer qualified health plans and stand-alone dental plans. **It can't sell life or disability insurance** (CMS). When a life benefit shows up "with" your health coverage, even on the same enrollment screen at work, it's a **separate policy or rider with its own rules, its own beneficiary, and its own end date.**

Where life-type benefits show up alongside health coverage

1. At work: basic group life, often with AD&D

- Many employers that offer health insurance also offer **group term life**, often with **AD&D** attached. Per the Texas Department of Insurance, a basic group policy is usually **one or two times your salary** or a set amount, and you usually **don't have to answer health questions** unless you want more than the basic amount.

- That's why people call it "guaranteed issue up to a limit." Extra (supplemental or voluntary) coverage above that limit may require health questions, often called **evidence of insurability**.
- **When you leave the job, the coverage typically ends** (TDI). **COBRA doesn't help here**: it continues health plans, and plans that provide only life or disability benefits aren't covered (U.S. Department of Labor).
- What can help: **conversion** (turning group coverage into an individual policy without health questions) and, if your plan offers it, **portability** (keeping group term coverage under a separate policy). Deadlines are short. Texas law, for example, gives 31 days to apply and pay, and AD&D and other extras don't have to convert. **Ask HR before your last day.** *(This is Lesson 2's work-life question.)*

2. AD&D: accidental death and dismemberment

- AD&D pays a **stated benefit if you die or lose a limb, your sight, or similar because of an accident** (NAIC). As a **rider** on a life policy, it adds an extra payment when death is accidental (TDI). As a **standalone** policy, **it pays only for accidents, never for sickness** (Maryland Insurance Administration).
- **What "accidental" usually leaves out:** policies define it narrowly. Common exclusions include death caused or contributed to by **illness or disease (or its treatment)**, **suicide or self-inflicted injury**, **drug overdose or intoxication**, **war**, **committing a crime**, and some **high-risk activities**. Many also require the death or loss to happen **within a set time after the accident**. Read your policy's own list.
- Why that matters: in CDC's 2024 data, **heart disease and cancer each caused more than three times as many U.S. deaths as accidents.** AD&D can be a helpful extra, but on its own it leaves the biggest gap.

3. Small death benefits bundled into health packages

Some association memberships, private (non-ACA) health packages, and supplemental health policies, such as **hospital or fixed indemnity**, **accident**, or **critical illness**, include or offer a **small life benefit or AD&D rider.** These are often modest amounts, may be **accident-only**, and **may end if you drop the package.** Remember Lesson 5: these supplemental policies sit **alongside** a health plan; they don't replace one.

4. Guaranteed issue and simplified issue life (often called "final expense")

- **Simplified issue** asks a short list of health questions, usually with no exam. **Guaranteed issue** asks **no health questions**, so you can't be turned down for your health, within the policy's age limits.
- The trade-off, per South Carolina's Department of Insurance: the insurer balances the risk by **charging higher premiums or limiting how much you can buy**, and over enough years you could **pay in more than the policy pays out.** New York's regulator notes these plans are usually **more expensive than a fully underwritten policy** for someone who qualifies at standard rates.
- **The graded (or "modified") death benefit:** many of these policies don't pay the full amount right away. **Commonly, for the first two years** (some policies use a different period, and New York describes "generally the first two or three policy years"), if death is from **illness**, the beneficiary gets **only a return of the premiums paid, often plus interest**, or a partial benefit. **Accidental death is often paid in full from the start** (Florida Department of Financial Services). After the graded period, the full face amount applies. Check your policy for the exact rule.

- Separate from that, most individual policies have a **two-year contestable period** and a suicide clause (TDI), which is one more reason to answer every application question completely and honestly.

5. Pre-existing condition look-backs on riders and supplemental policies

This is the "about 12 months" wait people often hear about. ACA health plans can't exclude pre-existing conditions (Lesson 5), but **some supplemental health policies and riders can, for a while.** They **look back** at a set period before coverage starts (for example, 6 or 12 months) and may not pay claims tied to conditions from that period during the **first stretch of coverage, commonly up to 12 months.** An NAIC model regulation, which states can adopt in their own form, limits these exclusions, for example to **12 months for hospital indemnity** and **6 months for specified disease policies**, unless a condition is specifically excluded by name. **Rules vary by state and policy**, so find the "pre-existing condition" definition in your own paperwork.

6. Living benefits: accelerated death benefits and critical illness lump sums

- An **accelerated death benefit** (often a rider, sometimes in group life too) pays **part of the death benefit while you're alive** after a qualifying event, such as a **terminal illness** (usually a life expectancy of six months to a year), a specified serious illness, or a long-term care need (Alabama Department of Insurance; TDI).
- What it costs you: the advance is **subtracted from what your beneficiaries get**, there may be a charge or interest reduction, and Alabama's regulator notes it **isn't a form of health insurance.** It can affect **Medicaid eligibility** and taxes, so ask a tax professional.
- A **critical illness** policy is health-side coverage that can pay a **lump sum when you're diagnosed** with a listed condition (NAIC). That's money for living, not a death benefit, although some policies add a small one. Check which you have.

Who this can help, and the honest limits

It can help if your health history makes **fully underwritten term life** hard to get or very expensive. Group coverage at work, conversion rights when you leave, simplified or guaranteed issue policies, and AD&D can each provide **some** coverage. Texas's regulator notes you might get group life even if you can't buy a policy directly.

The honest limits:

- **Low face amounts.** Often enough for final expenses, not to replace years of income.
- **Higher cost per $1,000 of coverage** than underwritten term, for people who could qualify for it.
- **Accident-only gaps.** AD&D doesn't pay for illness, which causes most deaths.
- **Waiting periods.** Graded benefits and pre-existing look-backs can mean little or no payout early on.
- **Coverage that ends** when the job or the health package ends. COBRA won't continue it.

So the useful question isn't "is this good?" It's **"what exactly does this pay, for what, and until when?"**

Questions to ask / where to look in your paperwork

Where to look	What to find
Benefits guide or enrollment summary (work)	Is there basic life? AD&D? Supplemental life? Who pays for it?
Certificate of insurance (group life)	The **"Conversion"** and **"Portability"** sections, and the deadline after you leave
Schedule of benefits or policy data page	The face amount, any AD&D amount, and any graded benefit table
"Exclusions and limitations"	How "accident" is defined; the pre-existing condition look-back and how long it applies
"Graded," "limited," or "modified" death benefit section	How long it lasts, what's paid during it, and whether accidents are covered in full
Riders page	Accelerated or living benefit triggers, the amount, and any cost
"Termination" or "When coverage ends"	Job ends, a certain age, or the health package is dropped
Beneficiary designation (HR portal or insurer)	Is it current?

Six questions for HR or the insurer:

1. Does this pay for **any cause of death**, or **accidents only**?
2. How much does it pay, and does it **reduce at a certain age**?
3. Is there a **waiting, graded, or pre-existing condition period**? How long, and what's paid during it?
4. What happens if I **leave my job or drop the health plan**? Can I convert or port it, and **by when**?
5. Who's my **beneficiary** right now?
6. Are there **living benefits**, and what triggers them?

Two quick notes

- **Taxes:** according to the IRS, life insurance proceeds paid to a beneficiary because of the insured person's death **generally aren't taxable income**. Interest on them is, and there are exceptions. *For your situation, ask a tax professional.*
- **A kindness for the people you leave behind:** if someone you love has died and you aren't sure what coverage they had, the NAIC's free **Life Insurance Policy Locator** helps beneficiaries search. Their old job's HR office is worth a call too.

Key takeaways

- **Health plans don't include life insurance**, and the Marketplace can't sell it. Life-type benefits that come "with" health coverage are **separate policies or riders.**
- **Work life and AD&D** usually need no health questions up to a limit, but **typically end when the job ends**, and **COBRA doesn't continue them.** Look for conversion or portability, and mind the deadline.
- **AD&D pays for accidents only.** Read how "accidental" is defined.
- **Guaranteed and simplified issue** policies can help when health makes underwriting hard, but they're **smaller and cost more per dollar of coverage**, and many have a **graded benefit, commonly for about two years.**
- **Some supplemental policies and riders exclude pre-existing conditions for a while, commonly up to about 12 months.** Check your own policy.
- Ask of every benefit: **what does it pay, for what, and until when?**

Next up → Lesson 7: Your next step.

Sources

- NAIC, Life Insurance consumer page (term and cash value, buying basics, review every few years, Policy Locator): content.naic.org/consumer/life-insurance.htm
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- IRS, Life Insurance & Disability Insurance Proceeds FAQ: irs.gov/faqs/interest-dividends-other-types-of-income/life-insurance-disability-insurance-proceeds/life-insurance-disability-insurance-proceeds

Worksheet · "My life-benefits inventory"

Pull out your benefits guide, health package paperwork, and any policies. Blanks and question marks are fine; they're your to-do list.

Step 1: What do I already have? (Skip any row you don't have. "Not sure" is a fine answer.)

Benefit	Amount	Any cause, or accident only?	Waiting or graded period?	Ends when?
Work: basic life				
Work: AD&D				
Work: extra life				
Health package or supplemental policy				
My own policy				
Other				

Step 2: Who would need help if my income stopped? (Words are fine.)

_____

Step 3: If I leave my job or drop a health package

- ☐ I've checked whether my work life coverage can be converted or ported
- ☐ The deadline to apply after I leave is: _____
- ☐ I know which benefits end with the job or package: _____

Step 4: Paperwork check

- ☐ I've found the "exclusions," "pre-existing condition," and "graded benefit" sections, or confirmed there aren't any
- ☐ My beneficiaries are current (check after marriage, divorce, or a birth)
- ☐ A trusted person knows where these papers are kept
- ☐ I'll review this again at open enrollment or my next job change: _____

This is a conversation starter, not a recommendation. Coverage, pricing, and eligibility depend on the insurer, the policy, and your state.

Lesson 7

Your Next Step

Reading time: about 3 minutes · Worksheet: your one-page plan, 10 minutes



The hook

"When is open enrollment?" "Can I get health insurance now?" "Is health insurance required?" You've made it to the last lesson, so let's turn everything you've learned into **one page you can actually use**, with real dates and free, official places to get help.

You're allowed to protect your health and your family without being treated like a transaction. One step at a time.

The lesson

Three questions before you choose health coverage

1. **Can I see my doctors?** (Lesson 4)
2. **What do I pay if I get sick?** (Lesson 1)
3. **Is this the right enrollment window?** (Lesson 2 and below)

And one for life insurance:

1. **Who would need help if my income stopped?** (Lesson 6)

If you can answer those four, you're no longer lost in the woods. You're reading the trail signs.

The real dates (checked Oct 29, 2026)

Marketplace coverage for 2027 (the 29 states that use HealthCare.gov to enroll. Not sure if yours is one? Check the state appendix):

Date	What happens
Nov 1, 2026	Open Enrollment starts. You can enroll, renew, or change plans
Dec 15, 2026	Last day to enroll or change plans for coverage starting **Jan 1, 2027**
Jan 1, 2027	Coverage starts for those who enrolled by Dec 15 and paid the first premium
Jan 15, 2027	Open Enrollment ends

Date	What happens
Feb 1, 2027	Coverage starts for those who enrolled Dec 16 – Jan 15 and paid the first premium

- If you already have a Marketplace plan and don't act by Dec 15, HealthCare.gov says you **may** be re-enrolled automatically, but not in every case. Either way, you should update your application so your savings are right for next year.
- The 22 states (including DC) that run their own Marketplace for 2027 may have **different dates**. The state appendix shows which kind yours is, and HealthCare.gov's state picker sends you to the right official site.
- After Jan 15, you can enroll or change plans only with a **Special Enrollment Period** (Lesson 2). **Medicaid and CHIP** accept applications any time of year.
- HealthCare.gov also notes a law change: starting Jan 1, 2027, **only people with certain immigration statuses qualify for savings** on Marketplace plans. If that might affect your household, read HealthCare.gov's immigration page carefully.

Medicare (generally for people 65+, and some younger people with disabilities): Medicare's Open Enrollment Period runs **Oct 15 – Dec 7** each year, for coverage starting Jan 1. It's a different window from the Marketplace's. Your first chance to sign up, the Initial Enrollment Period, is generally a 7-month window around your 65th birthday.

Job-based coverage: your employer sets its own open enrollment dates. Ask HR or watch for the benefits email.

"Is health insurance required?"

There's **no longer a federal tax penalty** for not having health coverage. HealthCare.gov says the fee no longer applies starting with the 2019 plan year. Some states may have their own rules, so check with your state's insurance department.

Free, official help (no sales pitch attached)

For	Who	How
Marketplace questions and applications	HealthCare.gov Marketplace Call Center	1-800-318-2596 (TTY 1-855-889-4325)
Medicare itself	Medicare	1-800-MEDICARE (1-800-633-4227), TTY 1-877-486-2048
Free, unbiased Medicare counseling	Your State Health Insurance Assistance Program (SHIP). Every state has one, and they aren't connected to any insurance company	Find yours at shiphelp.org
Questions or complaints about an insurer, agent, or policy	Your state insurance department	Find yours on the NAIC Insurance Departments map

Find your state

Every state and DC has its own insurance department, a free Medicare counseling program (SHIP), and either uses HealthCare.gov or runs its own Marketplace. We put all three for **all 50 states + DC** in one table:

Appendix: Find Your State

Or go straight to the official national directories, which are always the most current:

- **State insurance departments:** NAIC map, content.naic.org/state-insurance-departments
- **SHIP Medicare counseling:** shiphelp.org
- **HealthCare.gov or your state's own Marketplace:** healthcare.gov/marketplace-in-your-state

Phone numbers and directories checked Oct 29, 2026. Always double-check you're on an official site (Lesson 5).

Key takeaways

- Four questions do most of the work: **doctors, sick-day cost, enrollment window, who depends on me.**
- **2027 Marketplace Open Enrollment (HealthCare.gov states): Nov 1, 2026 – Jan 15, 2027.** Enroll by **Dec 15** for a **Jan 1** start. States with their own Marketplace may differ.
- **Medicare Open Enrollment: Oct 15 – Dec 7.** It's a separate window.
- There's **no federal penalty** for being uninsured, but check your state.
- **Free, unbiased help exists in every state:** the Marketplace Call Center, your state's SHIP for Medicare counseling, and your state insurance department. See the Find Your State appendix.

Thank you

Thank you for spending this time with us. Come back whenever life hands you a new insurance question; the course will be right here.

Sources

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- SHIP national directory (SHIP National Technical Assistance Center): shiphelp.org
- NAIC, Insurance Departments directory: content.naic.org/state-insurance-departments
- HealthCare.gov, The Marketplace in your state: healthcare.gov/marketplace-in-your-state
- Fern & Fine Print, Appendix: Find Your State (all 50 states + DC, with its own sources), at the end of this guide

Worksheet · My one-page plan

1. My situation in one sentence:

_____

2. My dates

What	Date
My next enrollment window opens	
The deadline I care most about	
(If coverage is ending) My coverage end date + 60 days	
The day I'll sit down and compare (put it in your calendar!)	

3. My four answers

Can I see my doctors? ☐ Yes ☐ Not yet checked → Lesson 4 worksheet

What do I pay if I get sick? Rough-year cost: $_____ → Lesson 1 worksheet

Is this the right window? ☐ Open Enrollment ☐ Special Enrollment ☐ Medicare ☐ Job-based

Who would need help if my income stopped? _____ → Lesson 6 worksheet

4. Documents I'll gather before I compare

- ☐ My current plan's Summary of Benefits and Coverage (if I have a plan)
- ☐ My must-keep doctors and prescriptions list (Lesson 4)
- ☐ A rough estimate of household income for next year
- ☐ Any notices from my plan, employer, or the Marketplace

5. Where I'll go for free, official help if I get stuck: _____

6. Confidence check, take two (compare with Lesson 0)

I can explain...	1	2	3	4	5
What a deductible is	☐	☐	☐	☐	☐
What an out-of-pocket maximum is	☐	☐	☐	☐	☐
The difference between an HMO and a PPO	☐	☐	☐	☐	☐
When I'm allowed to sign up for or change a plan	☐	☐	☐	☐	☐
Term vs. permanent life insurance	☐	☐	☐	☐	☐

Did a number move? That's you, getting less confused. Well done.

Appendix

Find Your State: Free, Official Insurance Help

Compiled and checked Oct 29, 2026. Phone numbers and websites change, so if one doesn't work, use the national directories below. They're always the most current source.

Start here: the national directories

What you need	Official directory	Phone
Your **state insurance department** (questions, complaints, checking whether a company or agent is licensed)	NAIC Insurance Departments map: content.naic.org/state-insurance-departments	(listed by state)
Free, unbiased **Medicare counseling** from your State Health Insurance Assistance Program (SHIP)	SHIP national directory: shiphelp.org	(listed by state)
Whether your state uses **HealthCare.gov or its own Marketplace**	HealthCare.gov, The Marketplace in your state: healthcare.gov/marketplace-in-your-state	Marketplace Call Center 1-800-318-2596 (TTY 1-855-889-4325)
Medicare itself	Medicare.gov	1-800-MEDICARE (1-800-633-4227), TTY 1-877-486-2048

How to read the table

- **Insurance department:** the website and phone come from the department's own entry on the NAIC map. Where NAIC lists a general consumer line (a hotline or consumer services number), we show it. Otherwise we show the department's **main line** as NAIC lists it, and the website will point you to its consumer help page. For Florida, the website and the toll-free helpline come from the Florida Department of Financial Services' own Consumer Services page; the 850 number is from NAIC.
- **SHIP:** the program name, phone, and website come from each state's page on shiphelp.org (the SHIP National Technical Assistance Center). Where a program website wouldn't load during our check, we link the state's shiphelp.org page instead of guessing.
- **2027 Marketplace:** from CMS, "States by Marketplace Type for Plan Year 2027." Marketplace names come from each state's page on HealthCare.gov. We don't link enrollment sites here; use HealthCare.gov's state picker to get to the right one.
- "In-state" means the number works only when called from inside that state.

All 50 states + DC

State	Insurance dept website	Insurance dept phone	SHIP (free Medicare counseling)	SHIP phone	SHIP website	2027 Marketplace
Alabama	aldoi.gov	(334) 269-3550 (main line)	Senior Health Insurance Program (SHIP)	1-800-243-5463	alabamaageline.gov/ship	HealthCare.gov
Alaska	insurance.alaska.gov	(907) 269-7900 (main line)	Senior Health Insurance Program (SHIP)	1-800-478-6065	Directory: shiphelp.org/ships/alaska	HealthCare.gov
Arizona	difi.az.gov	(602) 364-3100 (main line)	Arizona State Health Insurance Assistance Program (SHIP)	1-800-432-4040	azship.org	HealthCare.gov
Arkansas	insurance.arkansas.gov	(501) 371-2600 (main line)	Senior Health Insurance Information Program (SHIIP)	1-800-224-6330	insurance.arkansas.gov/consumer-services/senior-health	Own Marketplace on the federal platform: apply through HealthCare.gov
California	insurance.ca.gov	(800) 927-4357 (consumer hotline)	California Health Insurance Counseling and Advocacy Program (HICAP)	1-800-434-0222	aging.ca.gov/hicap	Own Marketplace: Covered California
Colorado	doi.colorado.gov	(303) 894-7499 (main line)	State Health Insurance Assistance Program (SHIP)	1-888-696-7213	doi.colorado.gov/insurance-products/health-insurance/senior-health-care-medicare	Own Marketplace: Connect for Health Colorado
Connecticut	portal.ct.gov/cid	(860) 297-3800 (main line)	Connecticut's Program for Health Insurance Assistance, Outreach, Information and Referral, Counseling, Eligibility Screening (CHOICES)	1-800-994-9422	portal.ct.gov/ADS-CHOICES	Own Marketplace: Access Health CT
Delaware	insurance.delaware.gov	(302) 674-7300 (main line)	Delaware Medicare Assistance Bureau	1-800-336-9500	insurance.delaware.gov/divisions/dmab	HealthCare.gov
District of Columbia	disb.dc.gov	(202) 727-8000 (main line)	DC State Health Insurance Assistance Program (SHIP)	202-727-8370	dacl.dc.gov/service/health-insurance-counseling	Own Marketplace: DC Health Link
Florida	myfloridacfo.com/division/consumers/contactus	1-877-693-5236 (consumer helpline, in FL) · (850) 413-3089	Serving Health Insurance Needs of Elders (SHINE)	1-800-963-5337	floridashine.org	HealthCare.gov
Georgia	oci.georgia.gov	(404) 656-2070 (main line)	Georgia State Health Insurance Assistance Program	1-866-552-4464	aging.georgia.gov/georgia-ship	Own Marketplace (pick your state at healthcare.gov/marketplace-in-your-state)
Hawaii	cca.hawaii.gov/ins	(808) 586-2790 (main line)	Hawaii State Health Insurance Assistance Program (SHIP)	1-888-875-9229	hawaiiship.org	HealthCare.gov
Idaho	doi.idaho.gov	(208) 334-4250 (main line)	Senior Health Insurance Benefits Advisors (SHIBA)	1-800-247-4422	Directory: shiphelp.org/ships/idaho	Own Marketplace: Your Health Idaho
Illinois	insurance.illinois.gov	(312) 814-2420 (main line)	Senior Health Insurance Program (SHIP)	1-800-252-8966	ilaging.illinois.gov/ship.html	Own Marketplace (pick your state at healthcare.gov/marketplace-in-your-state)
Indiana	in.gov/idoi	(800) 622-4461 (consumer services, in-state)	State Health Insurance Assistance Program (SHIP)	1-800-452-4800	medicare.in.gov	HealthCare.gov

State	Insurance dept website	Insurance dept phone	SHIP (free Medicare counseling)	SHIP phone	SHIP website	2027 Marketplace
Iowa	iid.iowa.gov	(877) 955-1212 (consumer complaints, in-state)	Senior Health Insurance Information Program (SHIIP)	1-800-351-4664	shiip.iowa.gov	HealthCare.gov
Kansas	insurance.kansas.gov	(785) 296-3071 (main line)	Senior Health Insurance Counseling for Kansas (SHICK)	1-800-860-5260	kdads.ks.gov/services-programs/aging/medicare-programs/senior-health-insurance-counseling-for-kansas-shick	HealthCare.gov
Kentucky	insurance.ky.gov	(502) 564-3630 (main line)	State Health Insurance Assistance Program (SHIP)	1-877-293-7447	Directory: shiphelp.org/ships/kentucky	Own Marketplace: Kynect
Louisiana	ldi.la.gov	(225) 342-5900 (main line)	Senior Health Insurance Information Program (SHIIP; the state page now calls it LaSHIP)	1-800-259-5300	ldi.la.gov/consumers/senior-health-shiip	HealthCare.gov
Maine	maine.gov/pfr/insurance/home	(207) 624-8475 (main line)	Maine State Health Insurance Assistance Program (SHIP)	1-800-262-2232	maine.gov/dhhs/oads/get-support/older-adults-disabilities/older-adult-services/ship-medicare-assistance	Own Marketplace: CoverME.gov
Maryland	insurance.maryland.gov	(410) 468-2000 (main line)	State Health Insurance Assistance Program (SHIP)	1-800-243-3425	aging.maryland.gov/Pages/state-health-insurance-program.aspx	Own Marketplace: Maryland Health Connection
Massachusetts	mass.gov/orgs/division-of-insurance	(617) 521-7794 (main line)	Serving Health Insurance Needs of Everyone (SHINE)	1-800-243-4636	mass.gov/health-insurance-counseling	Own Marketplace: Health Connector
Michigan	michigan.gov/difs	(517) 284-8800 (main line)	Michigan State Health Insurance Assistance Program (SHIP)	1-800-803-7174	michigan.gov/mdhhs/adult-child-serv/adults-and-seniors/acls/state-health-insurance-assistance-program	HealthCare.gov
Minnesota	mn.gov/commerce	(651) 539-1500 (main line)	Minnesota Aging Pathways	1-800-333-2433	mn.gov/aging-pathways	Own Marketplace: MNsure
Mississippi	mid.ms.gov	(601) 359-3569 (main line)	State Health Insurance Assistance Program (SHIP)	1-844-822-4622	mdhs.ms.gov/aging/finding-services-for-older-adults	HealthCare.gov
Missouri	dci.mo.gov	(573) 751-4126 (main line)	Missouri State Health Insurance Assistance Program (SHIP)	1-800-390-3330	missouriship.org	HealthCare.gov
Montana	csimt.gov	(406) 444-2040 (main line)	Montana State Health Insurance Assistance Program (SHIP)	406-444-4077	dphhs.mt.gov/sltc/aging/ship	HealthCare.gov
Nebraska	doi.nebraska.gov	(402) 471-2201 (main line)	Nebraska SHIP	1-800-234-7119	doi.nebraska.gov/consumer/senior-health	HealthCare.gov
Nevada	doi.nv.gov	(775) 687-0700 (main line)	Nevada Medicare Assistance Program (MAP)	1-800-307-4444	nevadacareconnection.org/care-options/types-of-services/healthcare/medicare-assistance-program-map	Own Marketplace: Nevada Health Link
New Hampshire	nh.gov/insurance	(603) 271-2261 (main line)	New Hampshire State Health Insurance Assistance Program (SHIP) – ServiceLink Resource Center	1-866-634-9412	servicelink.nh.gov/medicare/index.htm	HealthCare.gov
New Jersey	state.nj.us/dobi/index.html	(609) 292-7272 (main line)	State Health Insurance Assistance Program (SHIP)	1-800-792-8820	nj.gov/humanservices/doas/services/q-z/ship	Own Marketplace: Get Covered NJ

State	Insurance dept website	Insurance dept phone	SHIP (free Medicare counseling)	SHIP phone	SHIP website	2027 Marketplace
New Mexico	osi.state.nm.us	(855) 427-5674 (main line)	New Mexico ADRC – State Health Insurance Assistance Program (SHIP)	1-800-432-2080	aging.nm.gov/consumer-and-elder-rights/medicare	Own Marketplace: beWellnm
New York	dfs.ny.gov	(800) 342-3736 (consumer hotline)	Health Insurance Information, Counseling and Assistance (HIICAP)	1-800-701-0501	aging.ny.gov/health-insurance-information-counseling-and-assistance	Own Marketplace: New York State of Health
North Carolina	ncdoi.gov	(919) 807-6000 (main line)	Seniors' Health Insurance Information Program (SHIIP)	1-855-408-1212	ncdoi.gov/consumers/medicare-and-seniors-health-insurance-information-program-shiip	HealthCare.gov
North Dakota	insurance.nd.gov	(701) 328-2440 (main line)	State Health Insurance Counseling Program (SHIC)	1-888-575-6611	insurance.nd.gov/shic-medicare	HealthCare.gov
Ohio	insurance.ohio.gov	(614) 644-2658 (main line)	Ohio Senior Health Insurance Information Program (OSHIIP)	1-800-686-1578	insurance.ohio.gov/consumers/medicare/01-oshiip	HealthCare.gov
Oklahoma	ok.gov/oid	(405) 521-2828 (main line)	Senior Health Insurance Counseling Program (SHIP)	1-800-763-2828	Directory: shiphelp.org/ships/oklahoma	Own Marketplace on the federal platform: apply through HealthCare.gov
Oregon	dfr.oregon.gov	(503) 947-7980 (main line)	Senior Health Insurance Benefits Assistance (SHIBA)	1-800-722-4134	shiba.oregon.gov	Own Marketplace: ExploreHealthOR.gov (new for 2027)
Pennsylvania	insurance.pa.gov	(717) 787-7000 (main line)	PA MEDI – Pennsylvania Medicare Education and Decision Insight	1-800-783-7067	pa.gov/agencies/aging/aging-programs-and-services/pa-medi-medicare-counseling	Own Marketplace: Pennie
Rhode Island	dbr.ri.gov/insurance-overview	(401) 462-9520 (main line)	Senior Health Insurance Program (SHIP)	1-888-884-8721	oha.ri.gov/Medicare	Own Marketplace: HealthSource RI
South Carolina	doi.sc.gov	(803) 737-6160 (main line)	State Health Insurance Assistance Program (SHIP)	1-800-868-9095	aging.sc.gov/programs-initiatives/medicare-and-medicare-fraud	HealthCare.gov
South Dakota	dlr.sd.gov/insurance	(605) 773-3563 (main line)	Senior Health Information and Insurance Education (SHIINE)	1-888-854-5321	shiine.net	HealthCare.gov
Tennessee	tn.gov/commerce	(615) 741-2241 (main line)	State Health Insurance Assistance Program (SHIP)	1-877-801-0044	tn.gov/disability-and-aging/disability-aging-programs/tn-ship.html	HealthCare.gov
Texas	tdi.texas.gov	(800) 252-3439 (consumer help line)	Health Information, Counseling, and Advocacy Program (HICAP)	1-800-252-9240	hhs.texas.gov/services/health/medicare	HealthCare.gov
Utah	insurance.utah.gov	(801) 957-9200 (main line)	Senior Health Insurance Information Program (SHIP)	1-800-541-7735	daas.utah.gov/seniors	HealthCare.gov
Vermont	dfr.vermont.gov	(802) 828-3301 (main line)	State Health Insurance Assistance Program (SHIP)	1-800-642-5119	asd.vermont.gov/services/ship	Own Marketplace: Vermont Health Connect
Virginia	scc.virginia.gov	(804) 371-9741 (main line)	Virginia Insurance Counseling & Assistance Program (VICAP)	1-800-552-3402	dars.virginia.gov/aging/home-community/medicare-counseling	Own Marketplace (pick your state at healthcare.gov/marketplace-in-your-state)
Washington	insurance.wa.gov	(360) 725-7000 (main line)	Statewide Health Insurance Benefits Advisors (SHIBA)	1-800-562-6900	insurance.wa.gov/get-help-medicare	Own Marketplace: Washington Healthplanfinder

State	Insurance dept website	Insurance dept phone	SHIP (free Medicare counseling)	SHIP phone	SHIP website	2027 Marketplace
West Virginia	wvinsurance.gov	(304) 558-3386 (consumer services)	State Health Insurance Assistance Program (SHIP)	1-877-987-4463	Directory: shiphelp.org/ships/west-virginia	HealthCare.gov
Wisconsin	oci.wi.gov	(608) 266-3585 (main line)	State Health Insurance Assistance Program (SHIP)	1-800-242-1060	dhs.wisconsin.gov/benefit-specialists/medicare-counseling.htm	HealthCare.gov
Wyoming	doi.wyo.gov	(307) 777-7401 (main line)	State Health Insurance Assistance Program (SHIP)	1-800-856-4398	wyomingseniors.com	HealthCare.gov

Sources

- NAIC, Insurance Departments directory (state entries checked Oct 29, 2026): content.naic.org/state-insurance-departments
- SHIP National Technical Assistance Center, state SHIP pages (checked Oct 29, 2026): shiphelp.org
- CMS, States by Marketplace Type (FFM/SBM-FP/SBM) for Plan Year 2027: cms.gov/marketplace/in-person-assisters/training-webinars/training/marketplaces-map
- HealthCare.gov, state pages and "The Marketplace in your state" (Marketplace names; Oregon's 2027 change): healthcare.gov/marketplace-in-your-state
- Florida Department of Financial Services, Consumer Services contact page: myfloridacfo.com/division/consumers/contactus
- U.S. Department of Labor, A Worker's Guide to Health Benefits Under COBRA (Marketplace Call Center numbers): dol.gov/agencies/ebsa/about-ebsa/our-activities/resource-center/publications/workers-guide-health-benefits-cobra
- Medicare.gov, Joining a plan (1-800-MEDICARE, TTY): medicare.gov/basics/get-started-with-medicare/get-more-coverage/joining-a-plan